

Mental Status Examination

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Smart Health tower

- The purpose of the MSE is not simply to gather descriptive information, but to establish a foundation for **diagnosis, risk assessment, treatment planning, and ongoing follow-up.**
- Many psychiatric disorders—such as schizophrenia, bipolar disorder, major depression, anxiety disorders, personality disorders, and cognitive disorders—have clear, observable manifestations that can be detected through the MSE. Therefore, learning to perform a thorough and objective MSE is a core competency for all medical students and clinicians.

- Unlike laboratory or imaging tests, the MSE relies entirely on the clinician's **observation, interview skills, and clinical judgment.**
- It integrates both verbal and non-verbal cues, making it as much an art as a science.
- The MSE evaluates a series of domains including **appearance, behavior, speech, mood, affect, thought processes, thought content, perception, cognition, insight, and judgment.**
- These domains together create a coherent picture of the patient's mental state at that particular moment.

- Ultimately, the **MSE** is a vital tool that bridges clinical observation with diagnostic reasoning, allowing the psychiatrist to understand the patient's internal world and to make informed, evidence-based decisions about management.
- Mastery of the MSE is a foundational skill that every future doctor must acquire to competently evaluate and care for patients with mental health concerns.

Differences Between MSE and Physical Examination

- **1. Purpose**

- **MSE:** Assesses psychological, emotional, cognitive, and behavioural functioning.
- **Physical Exam:** Assesses biological and physiological systems.

- **2. Type of Data**

- **MSE:** Mostly observational and interpretive; based on interview.
- **Physical Exam:** Objective, measurable; uses instruments and hands-on techniques.

- **3. Method**

- **MSE:** Conducted through conversation, mental tasks, and clinician observation.
- **Physical Exam:** Uses inspection, palpation, percussion, auscultation.

Differences Between MSE and Physical Examination

- **4. Focus**

- **MSE:** Appearance, speech, mood, affect, thought, perception, cognition, insight, judgment.
- **Physical Exam:** Cardiovascular, respiratory, GI, neurological, musculoskeletal, skin.

- **5. Diagnostic Role**

- **MSE:** Identifies psychiatric disorders and risk (suicide, violence).
- **Physical Exam:** Identifies medical and surgical conditions.

- **6. Stability**

- **MSE:** Highly dynamic; can change hourly.
- **Physical Exam:** More stable; findings change gradually.

- **7. Required Skills**

- **MSE:** Strong interviewing, observation, understanding of psychopathology.
- **Physical Exam:** Technical examination skills, anatomical and physiological knowledge.

MAIN ELEMENTS OF THE MENTAL STATUS EXAMINATION

Appearance & Behavior

- grooming, clothing
- cooperation
- eye contact

Thought Content

- delusions
- obsessions
- suicidal ideation

Speech

- rate
- volume
- fluency

Perception

- hallucinations
- illusions

Mood & Affect

- mood
- range, appropriateness

Cognitive Functions

- orientation
- memory
- attention

Thought Process

- linear-logical
- goal-directedness

Insight & Judgment

- awareness of illness
- decision-making

I. Appearance & Behaviour

- **A. Appearance**

- **General physical description:** age, sex, ethnicity, body habitus
- **Hygiene & grooming:** well-groomed, disheveled, malodorous
- **Clothing:** appropriate for weather? mismatched? bizarre?
- **Posture & gait:** steady, shuffling (Parkinson's), ataxic (alcohol), slumped (depression)
- **Eye contact:** normal, poor (depression, autism), intense (mania, paranoia)

Clinical examples:

- Schizophrenia:**

- poor grooming, inappropriate clothing, malodorous

- Major depressive episode:**

- minimal grooming, slumped posture

- Manic episode:**

- flamboyant clothing, excessive makeup

- OCD:**

- overly neat, meticulous grooming

B. Behaviour

- Observe from first seconds:
- **Psychomotor activity:** agitation vs retardation
- **Gestures:** fidgeting, hand wringing
- **Motor abnormalities:**
 - Catatonia: stupor, mutism, waxy flexibility
 - Echopraxia: copying movements
 - Stereotypies: repetitive non-goal-directed movements
- **Level of cooperation:** cooperative, guarded, hostile
- **Social interactions:** intrusive, withdrawn, overfamiliar
- **Physical movements signalling internal stimuli:** looking around, laughing to self

Clinical examples:

- Mania:**

- pacing, dancing, overfamiliar

- Delirium:**

- pulling lines, restless

- Severe anxiety:**

- tremors, sweating, fidgeting

- Intoxication:**

- unsteady gait, lurching

- Catatonia:**

- posturing, echolalia

II. Speech

- Speech is a window into thought.

- **Parameters to Assess**
- **Rate:** slow, normal, rapid
- **Volume:** whispered, loud, variable
- **Fluency:** stuttering, slurred, hesitant
- **Tone:** monotonous (Parkinson's), melodious (mania)
- **Productivity:** spontaneous vs sparse
- **Articulation:** dysarthria (alcohol, stroke), aphasia (neurological)

Clinical examples:

- **Abnormalities**

- **Pressured speech:** hallmark of mania; patient is unable to be interrupted
- **Poverty of speech (Alogia):** schizophrenia negative symptoms
- **Delayed responses:** depression
- **Cluttering:** ADHD
- **Stilted speech:** autism spectrum

III. Mood & Affect

- **A. Mood (*subjective*)**
- Ask open questions:
- “How have you been feeling emotionally?”
- “What is your general mood LATELY?”
 - **Descriptors**
 - Euthymic
 - Depressed
 - Anxious
 - Irritable
 - Euphoric or expansive
 - Angry
 - Dysphoric
 - Apathetic

Clinical examples:

- **Depression:**

- sad, empty, hopeless mood

- **Anxiety disorders:**

- nervous, worried, “on edge”

- **Mania:**

- unusually cheerful, euphoric, irritable if frustrated

- **Borderline personality disorder:**

- rapidly shifting moods

B. Affect (*objective*)

- **Evaluate:**
- **Range**
 - Full
 - Constricted
 - Blunted
 - Flat
- **Reactivity:**
 - Normal
 - Reduced
 - Labile
- **Appropriateness:** congruent vs incongruent
- **Intensity:** heightened, subdued, shallow

Clinical examples:

- Depression:**

- constricted or blunted

- Schizophrenia:**

- flat, incongruent

- Mania:**

- expansive, intense

- PTSD:**

- restricted affect, emotional numbing

IV. Thought Process

- **Describe how ideas are connected.**
- **Normal**
- Linear
- Logical
- Goal-directed

Clinical examples:

- **Circumstantiality:** long, overly detailed, but eventually reaches the point
- **Tangentiality:** never reaches the point
- **Flight of ideas:** very rapid, connections based on puns / rhymes (mania)
- **Loose associations:** illogical shifting (schizophrenia)
- **Thought blocking:** sudden halt mid-sentence
- **Derailment:** ideas that veer off track
- **Clang associations:** word associations based on sound
- **Perseveration:** repeating same idea
- **Neologisms:** invented words
- **Word salad:** incoherent mixture (severe psychosis)
- **Case demonstration for students:**
 - Give them a sample patient dialogue and ask them to identify the abnormality.

V. Thought Content

- **Core categories:**
- **Delusions:** fixed false beliefs
 - Persecutory
 - Referential
 - Grandiose
 - Somatic
 - Nihilistic
 - Control (thought insertion, withdrawal, broadcasting)
- **Obsessions:** intrusive unwanted thoughts
- **Phobias:** irrational fears with avoidance
- **Overvalued ideas:** exaggerated beliefs (anorexia, hypochondriasis)
- **Suicidal ideation:**
 - Passive
 - Active
 - Plans
 - Means
 - Previous attempts
- **Homicidal ideation:** intent or plans
- **Preoccupations:** guilt, health, religion

Clinical examples:

- “My heart stopped beating” → nihilistic delusion
- “Someone implanted a chip in my brain” → delusion of control
- “If I don’t clean the room perfectly, something bad will happen” → obsession + magical thinking

VI. Perception

- **Hallucinations**
- Perception without an external stimulus.
- **Auditory:** most common → schizophrenia
 - 2nd-person voices
 - Running commentary
 - Command hallucinations
- **Visual:** more common in organic states
 - Delirium
 - Charles-Bonnet syndrome
 - Substance intoxication / withdrawal
- **Tactile:** formication in cocaine use
- **Olfactory:** temporal lobe epilepsy
- **Gustatory:** rare; often organic etiology

VI. Perception

- **Illusions**
- Misinterpretation of a real object.
Seen in:
 - Anxiety
 - Grief
 - Delirium
 - PTSD (hypervigilance)
- **Depersonalization & Derealization**
- Common in anxiety, PTSD, dissociation.
- Feeling unreal
- Feeling world is unreal

VII. Cognitive Functions

- **A. Level of Consciousness**
 - Alert
 - Drowsy
 - Stuporous
 - Comatose
- **B. Orientation**
 - Ask the 4 domains:
 - Time
 - Place
 - Person
 - Situation
- **C. Attention & Concentration**
 - Serial 7s
 - Months backward
 - Digit span
- **Impaired in:** delirium, intoxication, depression.

- **D. Memory**
 - Immediate: repeat numbers
 - Short-term: recall 3 objects after 5 minutes
 - Long-term: personal history
- **E. Language**
 - Naming
 - Word repetition
 - Following commands
 - Fluency
 - Reading/writing (if needed)
- **F. Visuospatial ability**
 - Clock drawing
 - Copying shapes (interlocking pentagons)
- **G. Abstract thinking**
 - Ask proverbs or similarities.
 - Concrete thinking → schizophrenia, dementia
 - Good abstraction → normal

VIII. Insight

- Levels of insight:
- **No insight:** denies illness entirely
- **Partial:** acknowledges symptoms but attributes externally
- **Full:** understands illness and need for treatment
- **Clinical relevance**
- Psychosis: poor
- Mania: absent
- OCD: usually intact
- Depression: intact
- Ask:
- “Do you believe you have a medical or psychological condition?”
- “Why are you in the hospital?”
- “Do you think medications can help?”

IX. Judgment

- **Three domains**
- **Personal judgment:** life decisions
- **Social judgment:** interactions
- **Test judgment:** hypothetical scenarios
- Ask:
 - “If you found a wallet on the street, what would you do?”
 - “If your house caught fire, what would you do first?”
- **Impaired in:**
 - Substance use
 - Mania
 - Dementia
 - Personality disorders
 - Psychosis

X. Risk Assessment

- A structured evaluation of the potential for harm
- Essential part of every psychiatric assessment
- Helps guide clinical decisions (e.g., admission vs discharge)
- Dynamic process — risk changes with time, stress, and treatment
- Based on:
 - Clinical interview
 - Mental Status Examination
 - Collateral information
 - Clinical judgment

Major Categories of Risk

- **Suicide risk**
- **Non-suicidal self-injury (NSSI)**
- **Violence/Homicide risk**
- **Self-neglect**
- **Vulnerability (abuse, exploitation)**
- **Risk arising from mental or physical illness (e.g., delirium, psychosis)**

Suicide Risk Assessment : Core Elements

- Suicidal Ideation**

- Frequency, intensity, duration

- Intent**

- Passive (“I wish I could disappear”)
- Active (“I want to kill myself”)

- Plan**

- Method
- Lethality
- Preparation

- Access to Means**

- Medications, weapons, high places, poisons

- Past Attempts**

- Strongest predictor of future attempts

- Current Stressors**

- Protective Factors**

Warning Signs for Suicide

- **Hopelessness or helplessness**
- **Suddenly becoming calm after intense distress**
- **Giving away possessions**
- **Writing goodbye letters**
- **Social withdrawal and isolation**
- **Increased substance use**
- **Talking directly or indirectly about dying**

Questions to Ask (Suicide Risk)

- “Have you had thoughts of harming yourself?”
- “Do you intend to act on these thoughts?”
- “Have you made a plan?”
- “Do you have access to the means?”
- “Have you attempted suicide before?”
- “What is stopping you from acting on these thoughts?”
- “What helps you stay safe?”

Protective Factors

- Strong family and social support
- Religious or cultural beliefs discouraging suicide
- Children or dependents
- Fear of death
- Sense of responsibility
- Future goals and hope
- Good therapeutic relationship

SAD PERSONS Scale for Suicide Risk Assessment

- **S – Sex**
 - Male gender → higher risk
- **A – Age**
 - <19 or >45 → higher risk
- **D – Depression**
 - Clinical depression, hopelessness, sadness

SAD PERSONS Scale for Suicide Risk Assessment

- **P – Previous Attempt**
 - Strongest predictor of future suicide
- **E – Ethanol / Substance Use**
 - Alcohol or drug abuse → disinhibition, impulsivity
- **R – Rational Thinking Loss**
 - Psychosis, severe anxiety, intoxication → impaired judgment
- **S – Social Support Lacking**
 - Isolation, recent losses, poor family support
- **O – Organized Plan**
 - Detailed method, preparations, access to means
- **N – No Spouse**
 - Single, divorced, or widowed → higher risk
- **S – Sickness**
 - Chronic illness, pain, disability, terminal disease

SAD PERSONS Scale for Suicide Risk Assessment

- **Scoring**
- **0–4:** Low risk
- **5–6:** Moderate risk → consider close monitoring
- **7–10:** High risk → urgent psychiatric intervention

Violence / Homicide Risk

- Assess for:
 - History of aggression or violent behavior
 - Command hallucinations instructing violence
 - Paranoid or persecutory delusions
 - Impulsivity or inability to control anger
 - Intoxication or substance withdrawal
- Access to weapons
- Specific targets or threats

Self-Neglect

- Indicators of risk:
 - Poor personal hygiene
 - Malnutrition or dehydration
 - Medication refusal
 - Unsafe or unsanitary living conditions
 - Failure to seek medical attention
- Seen in:
 - Severe depression
 - Psychosis
 - Dementia
 - Intellectual disability

Vulnerability & Safeguarding

- Patients may be at risk of:
 - Physical, sexual, or emotional abuse
 - Domestic violence
 - Financial exploitation
 - Neglect by caregivers
 - Human trafficking
 - Elder abuse
- Special groups:
 - Elderly
 - Children
 - Individuals with cognitive impairment
 - Women during pregnancy or postpartum
 - Refugees or displaced individuals

Levels of Risk

- **Low Risk:**
 - Passive suicidal thoughts
 - No plan, no intent
 - Strong protective factors
- **Moderate Risk:**
 - Suicidal ideation with risk factors
 - No immediate plan or intent
- **High Risk:**
 - Clear plan or intent
 - Access to means
 - Limited protective factors
- **Imminent Risk:**
 - Strong intent + means + preparation
 - Severe agitation or psychosis
 - Requires emergency action

Management Based on Risk Level

- **Low:**
 - Outpatient care
 - Psychoeducation
 - Therapy
 - Follow-up appointments
- **Moderate:**
 - Safety plan
 - Family involvement
 - Increased monitoring
 - Removal of means
- **High:**
 - Urgent psychiatric admission
- **Imminent:**
 - Emergency hospitalization
 - Continuous supervision
 - Crisis intervention

Safety Planning

- A structured plan including:
 - Identification of personal warning signs
 - Internal coping strategies
 - Social support networks
 - Crisis contacts
 - Removing access to means (medications/weapons)
 - Emergency services
- Safety plan must be:
 - Written
 - Shared with patient
 - Reviewed regularly

Integrating Risk with Mental Status Examination

- **Mood:** severe depression → suicide risk
- **Thought Content:** persecutory delusions → violence risk
- **Perception:** command hallucinations → risk of harm
- **Cognition:** delirium → accidental harm, wandering
- **Insight:** poor insight → high risk of noncompliance
- Always link symptoms → to → risk.